

جوستاف روسي انترناسيونال - مصر (EPE) * طلب علاج أول مرة / متردد

إسم المريض رابعياً : فاطمه السيد مصطفى محمد

العنوان : ش الحسينى

الحي / المدينة : منشييه ناصر

المحافظة : القاهرة

تاريخ تسجيل الإستمارة : 06-10-2026

رقم البطاقة : 28101270101123 رقم التلفون : 01102325049

العمر(تاريخ الميلاد) : 27-01-1981 المهنة : ربه منزل

الإقرار

أقر انا الموقع أدناه انني لا اتمتع باي نظام علاج مثل:- التأمين الصحي-القوات المسلحة-الشرطة-
الهيئات-المؤسسات او اي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة
نفقات العلاج في حالة ثبوت غير ذلك

الإخصائي الاجتماعي (-----) التوقيع: (-----)

المقرر بما فيه
التوقيع: (-----)

التشخيص الإكلينيكي : سرطان بالثدي (مبادرة)

نتائج الأبحاث : (مرفق)

التشخيص النهائي : سرطان بالثدي (مبادرة)

خطه العلاج : علاج مبادرة سرطان الثدي العلاج المساعد لوقاية العظام للمرضي
الذين يتلقوا علاج هرموني حمض الزيلودرونيك [14157]
التعليق على التشخيص : -----

تقرير اللجنة الثلاثية المتخصصة

الاسم: د/محمد کیلانی

الاسم: د/ندی حسن

الاسم: د/نرمين رمضان

التوقيع:  التوقيع: التوقيع:

تتعمد صحة البيانات المرفقة
رئيس اللجنة و مدير
المستشفى.

أطلعت اللجنة علي لائحة العلاج الخاصة بالمجالس وتلتزم جهة العلاج بلائحة المجالس الطبية. كما تقر اللجنة بأن البيانات المدونة في هذا النموذج

تتعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض بالمستشفى قبل الصرف.

أعضاء اللجنة

----- / ۲ ----- / ۲ ----- / ۲. | ----- / ۲. |

قرار اللجنة

التشخيص النهائي :

خطة العلاج :

المستشفى :

جـهة العلاج :-

ملاحظات : ---

ختم

شعار الجمهورية

مدير عام

مديرية الشؤون الصحية

رئیس

المجلس الطبي المتخصص

--

Name: فاطمة السيد مصطفى محمد National ID: 28101270101123	
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تقرير طبي

السيد
تحية طبية وبعد,,

إيماء للطلب المقدم من	بشأن تقرير طبي للمريض
بقسم:	رقم طبي:

نتشرف بأن نوضح فيما يلي تقريراً عن حالته
--

تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd	تاريخ وساعة الخروج : PM 01:17 05-07-2026
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وبعد الكشف والفحوصات والمتابعة وجد أنه
--

المریضة تعاني من ورم غير حميد بالثدي و تحتاج الي Zoledronic acid for bone protection during hormonal therapy
--

التاريخ : 05-07-2026	الوقت : -- --:--
	الطبيب المعالج : د/شيماء لاشين



فاطمة

السيد مصطفى محمد حسن

١٢ ش الصفي

مشرية ناصر - القاهرة

٢٨١٠٩٢٧٠٩٠٩١٩٢٢

KM7122202



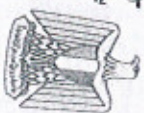
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بدون عمل

مشرية

انثى



الطاقة سارية حتى ٢٠٣١/٠٤/٠٥

١



03/25/2026

MDT FORM

Name:	فاطمة السيد مصطفى محمد	MRN:	8701
Age:	45 Y	ID:	28101270101123
Menopausal Sta	PREMENOPASUAL	Phone:	01102325049
NRS SCORE:		Marital Status:	MARRIED
BMI:		Family History:	-VE
CO Morbidity:		Reason Of MDT:	

Clinical Data:

BX: IDC GII
ER: 5/8 PR: 8/8 HER2: +3 KI67:

Sonomammography:

Mets Workup:

DYNAMIC MR-MAMOGRAPHY 1/1/2026
LT SIDED BREAST MASS BRIAD V +PATHOLOGICAL AXILLARY LN
TWO IRREGULAR SOILD MASSES ARE SEET AT 12 AND 3 OCLOC LT
2.2*1.8AND 4.1*3.1 CM
MULTIPLE LT AXILLARY LN WITH THICKENED CORTEX LIKELY PATHOLOGICAL
RT BREAST BRIAD II

PET CT 20/1/2026
LT BREAST LESION 6.1*3.2 CM SUV 22.3 AT 3 OCLOCK
2.6*2.6 CM AT 12 OCLOC SUV 13.8
LN
LT AXILLARY LN SUV 10.5 1.7*1.4 CM
SMALL RT AXILLARY LN SUV 1.2 1 CM
MEDIATINAL LN SUV 10.1 MEASURES 1.1 CM
LIVER HFL 3.6*3.3 CM SUV 21.4
FEW OSSEOUS LESIONS INVOLVING FEW VERTEBRA +RT ILIAC BONE SUV 4.3
AND SACRUM SUV 6.4 IN SACRUM
LV4 SUV 5.8
BULKY UTERUS SUV 6.6 AND RT ADENXAL CYSTIC LESION SUV 7.8 MEASURES
2.3*2.2 CM FOR SONOGRAPHIC CORRELATION

MRI PELVIS 10/2/2026 FREE

History:

DENOVO METSTATIC BREAST CANCER TO LN +BONE+LIVER

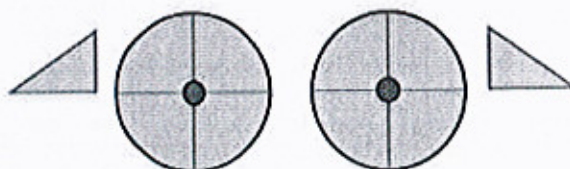
Recommendations:

MDT Outcome & Plan

Start 1st line Taxol/Herceptin with close monitoring of response.

R

L



T N M

SIGNATURE

MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY	Nutrition	Psychiatric
د/ شيماء لائين	د/ امليه عيسوي	د/منى زكي			

Name: فاطمه السيد مصطفى محمد		
MR. : 8701		
Detailed Medical Report		
		السيد
تحتية طبية وبعد,,		
	بشأن تقرير طبي للمريض	إيماء للطلب المقدم من
	رقم طبي:	بقسم:
ننشرف بأن نوضح فيما يلي تقريراً عن حالته		
	تاريخ وساعة الخروج :	تاريخ وساعة الدخول :
وبعد الكشف والفحوصات والمتابعة وجد انه 45 YEARS OLD FEMALE PREMENOPASUAL PT WITH DENOVO METSTATIC BREAST CANCER TO LIVER+LN AND BONE IDC GII ER 5/8 PR 8/8 HER2+3 DYNAMIC MR-MAMOGRAPHY 1/1/2026 LT SIDED BREAST MASS BRIAD V +PATHOLOGICAL AXILLARY LN TWO IRREGULAR SOILD MASSES ARE SEET AT 12 AND 3 OCLOC LT 2.2*1.8AND 4.1*3.1 CM MULTIPLE LT AXILLARY LN WITH THICKENED CORTEX LIKELY PATHOLOGICAL RT BREAST BRIAD II PET CT 20/1/2026 LT BREAST LESION 6.1*3.2 CM SUV 22.3 AT 3 OCLOCK 2.6*2.6 CM AT 12 OCLOC SUV 13.8 LN LT AXILLARY LN SUV 10.5 1.7*1.4 CM SMALL RT AXILLARY LN SUV 1.2 1 CM MEDIATINAL LN SUV 10.1 MEASURES 1.1 CM LIVER HFL 3.6*3.3 CM SUV 21.4 FEW OSSEOUS LESIONS INVOLVING FEW VERTEBRA +RT ILIAC BONE SUV 4.3 AND SACRUM SUV 6.4 IN SACRUM LV4 SUV 5.8 BULKY UTERUS SUV 6.6 AND RT ADENXAL CYSTIC LESION SUV 7.8 MEASURES 2.3*2.2 CM FOR SONOGRAPHIC CORRELATION MRI PELVIS 10/2/2026 FREE FOR MDT C1 HERCEPTIN IV AFTER ECHO + WEEK 1 TAXOL AFTER LABS WEEK 2 TAXOL , WEEK 3 TAXOL >>> PHARMACY PHARMACY NOTE: ECHO7/4 68% WT 87 KG HT 155CM S.A 1.9 Pharmacy VIsit Wt: 87, Ht: 155, BSA 1.9 For W3 Taxol 150 mg Labs 7/4 ... Transaminases 2 folds, for liver support, otherwise accepted prem left breast cancer ct4 n2 m1 met to liver bone idc2 er5/8 pr8/8 her23+ initial pet ct bilobar hfl largest seg 5 suv 21 m 3.6x3.3 multiple osseous lesions vertebral few ,rt iliac bone rt adnexal cyst suv 6 but mri pelvis free.on exam left breast mass 8x5 cm left ax ln 2x2 for dental ex for w4 ,5,6 taxol 150 c2 herceptin 440ef68 Pharmacy VIsit For W 5 Taxol 150 mg Labs 28/4 ... accepted on exam left breast mass 4x4 left ax ln 2x2 cc grade 1 diarrhea bony pain for w7 taxol 150 c3 herceptin 440 dental ex pending w8,9 ph Pharmacy Visit For W8 Taxol 150 labs 19/5 ... accepted Pharmacy Visit For W9 Taxol 150 labs 19/5 ... accepted dental exa pending for c4 herceptin 440 and w10 taxol 150 mg .w11,12 ph new cts pending and mri brain Pharmacy Note For C4 Herceptin 440 mg W10 isn't due until 9/6		
تحريرا في ,		
	الوقت :	19-09-2026 التاريخ :
	dina makram الطبيب المعالج :	
يعتمد ,		

Pharmacy Visit For w11 taxol Patient recieved W10 at 9/6/2026

Pharmacy Visit For W12 Taxol 150 mg Labs 6/6 ... accepted

FOR W13 TAXOL + C5 HERCEPTIN IV AFTER LABS W14 and w15 pharmacy 7/7 14/7 for CT CAP + bone scan to be compared with baseline PET CT for new echo before next c6 her

Pharmacy Visit BSA: 1.9, Wt: 88, Ht: 155 For W14 Taxol 150 mg Labs 29/6 .. accepted

new cts pending ,bs pending on exam left breast induration decreased for w16 taxol 150 c6 herceptin 440 ef 60%21/7 ca vit d

45 Premenopausal left breast IDC G2, cT4N2M1 (liver, bone), ER 5/8, PR 8/8, HER2 3+. MRI/PET-CT (Jan 2026): multifocal left breast disease with axillary/mediastinal nodes, liver and bone metastases. MRI pelvis (10/02/2026): free. patient complains of disequilibrium and on and off headache MRI brain and ct cap requested today for week 17 taxol (g1 pN) labs accepted from last cycle

MRI to be done this thursday and CT CAP requested for w18 taxol, labs 21/7 accepted w19 pharmacy w new labs + w20 pharmacy

w19 taxol, labs accepted (didn't receive cycle last week) + c7 herceptin iv, w20 + 21 pharmacy pt to bring CT CAP next clinic visit, MRI brain free of any SOLs

Pharmacy Visit BSA: 1.9, Wt: 88 For W 20 Taxol 150 mg Labs 18/8 ... accepted

cts on 29/8/2026 : left breast nodule ,multiple bilobar hfl largest seg 5 largest 18x11(pet ct initially bilobar hfl largest seg 5 3.6x3.3), few rt iliac bone l1 l2 vertebral bodies sclerotic (pet ct few vertebral sacrum rt iliac) . sm partial response mri brain ischemic foci rd for comparison then mdt (to st ht with herceptin) on exam left uoq mass 4x4 grade 2 pn for c8 herceptin 440 w22 taxol 125 (15% dr)

metastatic breast cancer on tramadol 50 q 8h gaptin 600 at night will add paracetamol 500 q 8h



S C A N

NV-1

Name: *Fatma Al Sayed Moustafa*
Referred by: *Prof. Hospital Cleopatra*

ID: 5003952
Date: 01/01/2026

High Field (1.5T) Dynamic MR-Mammography of Both Breasts

Technique :

- Axial T2 (plate 1). Axial STIR (plate 2).
- Sagittal postcontrast 3D TFE of the right breast (plate 3).
- Sagittal postcontrast 3D TFE of the left breast (plate 4).
- Dynamic multiphase post-contrast study done in 8 minutes with MIP reconstruction (plate 5). Time intensity curves (plate 6).

Findings:

- Bilateral glandular condensation and evidences of left sided non homogenous glandular enhancement.
- Two irregular solid masses are seen at 12 and 3 O'clock position of the left breast (2.2x1.8 and 4.1 x 3.1 cm in diameters) . The masses show low signals in T2 and STAIR. The smaller mass shows irregular contrast enhancement, with diffuse non mass enhancement of the left breast at the site of the larger mass , the kinetic data obtained from the masses indicate malignant pattern , continuous rise followed by washout (type 3 curves) .
- Thickened and enhanced left breast skin is noted.
- No right breast masses and no architecture distortion.
- There are multiple left axillary LNs with thickened cortex likely pathological.

OPINION :

- Malignant left sided breast masses BIRADS V, with pathological left axillary LNs.
- Right sided fibroadenosis BIRADS II.

Prof. Faten-Kamel, MD

AMG
ALFA MEDICAL GROUP

A MEMBER OF ALFA MEDICAL GROUP

الفاسكان

16171

Patient Name : الأستاذ / فاطمه السيد مصطفى	Request No. : 13004962
Patient No. : 28101270101123	Request Date : 09/02/2026
Path No. : SR-222/26	Verification Date : 18/02/2026
Sex : Female	Financial Category : جوسفاف روسي - نفقة الدولة
Age : 45 Y	Department : Out Patient
Referreing Doc :	Registered by : Mrs.Mayada Hegazy

Pathology Report

Clinical Data: Left breast mass.

Nature of Specimen: Referred block & slides (Tru-cut biopsy).

Gross Examination:

One referred block labelled by (108-26). Together with, four referred slides including 1 H&E stained slide and 3 immunostained slides for (ER, PR, HER2neu).

Microscopic Examination:

- * Histological type: Invasive duct carcinoma.
- * Grade 2 (differentiation 3, nuclear pleomorphism 2, mitosis 2).
- * DCIS: 10% Intermediate grade ductal carcinoma in situ (Solid pattern with comedo type necrosis & microcalcification).
- * Angioinvasion: Absent.
- * Perineural invasion: Absent.
- * Microcalcification: Present.

* IPT:

- ER: Intermediate nuclear reaction in 20% of tumor cells (score 5/8).
- PR: Strong nuclear reaction in 80% of tumor cells (score 8/8).
- Her2: Strong complete membrane reaction in more than 10% of tumor cells (score 3+).

Diagnosis:

Left breast mass, Referred block & slides (Tru-cut biopsy):

- Invasive duct carcinoma, Grade 2.
- 10% Intermediate grade ductal carcinoma in situ.

IPT:

- ER: Positive (score 5/8).
- PR: Positive (score 8/8).

Verified by :

Dr.Duaa Mohamed

Printing Date : 08/03/2026 10:09:06

Page 1 of 2

Printing By : .marwa mohamed youss



غرفة رقم (4) - الدور الأول - مبنى رقم (987) - كورنيش النيل - مصر القديمة - القاهرة - مصر
Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadema - Cairo, Egypt

Patient Name	: الأستاذة / فاطمة السيد مصطفى	Request No.	: 13004962
Patient No.	: 28101270101123	Request Date	: 09/02/2026
Path No.	: SR-222/26	Verification Date	: 18/02/2026
Sex	: Female	Financial Category	: جوماتف روسي - نفقة الدولة
Age	: 45 Y	Department	: Out Patient
Referring Doc	:	Registered by	: Mrs.Mayada Hegazy

Pathology Report

- Her2: Positive (score 3+).

* Scoring system: "ER, PR"

ASCO-CAP ER/PR Test Guideline Recommendations: proportion of cells staining (0, no staining; 1, >1%; 2, between 1% and 10%; 3, between 11% and 33%; 4, between 34% and 66%

and 5, between 67%-100% of the cells staining). Intensity of positive tumor cells (0, none; 1, weak; 2, intermediate; and 3, strong). Total score from 0 to 8.

* Scoring system: "Her2"

Reference: ASCO-CAP Her2 Test Guideline Recommendation. IHC 0 (Null): as defined by no staining observed. IHC 0 (Ultra-low) as defined by membrane staining that is incomplete and is faint/barely perceptible and within < 10% of the invasive tumor cells. IHC 1+: as defined by incomplete membrane staining that is faint/barely perceptible and within > 10% of the invasive tumor cells.

IHC 2+: based on circumferential membrane staining that is incomplete and/or weak/moderate and within >10% of

the invasive tumor cells or complete and circumferential membrane staining that is intense and within < 10% of the invasive tumor cells. IHC 3+: based on circumferential membrane staining that is complete, intense > 10% of tumor cells.

Signature

Dr. Walaa Ibrahim Ghoneim.

Dr. Heba El-Shafeey.

Verified by :

Dr.Duaa Mohamed

Printing Date : 08/03/2026 10:09:06

Page 2 of 2

Printing By : .marwa mohamed youss



غرفة رقم (4) - الدور الأول - مبنى رقم (987) - كورنيش النيل - مصر القديمة - القاهرة - مصر
Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadema - Cairo, Egypt

ELSAYYED MOSTAFA, FATMA Patient ID: 10110226002215 DOB: 27-Jan-1981 Date: 10-Feb-2026
Patient Lastname: ELSAYYED MOSTAFA
Patient Firstname: FATMA
HIS Patient ID: 10110226002215
RIS Patient ID: 381206
Date of Birth: 27/01/1981

Procedure: 15722, MRI Pelvis with Contrast

Medical Question:

Clinical Comment: زيادة الجراحة

===== REPORT TEXT =====

MRI EXAMINATION OF THE PELVIS

MRI Findings:

- § No adnexal cystic or solid masses are seen only dominant right ovarian follicle
- § Mild diffuse thickening of the uterine junctional zone is noted likely mild diffuse adenomyosis
- § Normal appearance of the rectum and ischiorectal fossa on both sides.
- § Smooth filling of the urinary bladder with no mural thickening or diverticular outpouching.
- § Normal marrow signal pattern of the pelvic bones.

===== REPORT TEXT ENDE =====

جوسلاف
رونيديني
مستشفى جامعة القاهرةالأخت
أ. ي. ٩مبادرة رئيس الجمهورية
لدعم صحة المرأة المصريةمستشفى جامعة
القاهرة
الجديد

09/30/2026

MDT FORM

Name:	نظرة لبنو مصطفى محمد	MRN:	8701
Age:	46 Y	ID:	28101270101123
Menopausal Sta	prem	Phone:*	0115238524
NRS SCORE:		Marital Status:	
BMI:		Family History:	
Gender		Reason Of MDT:	
CO Morbidity:		mbc triple positive with RD for maintenance hormonal tt	
Surgical History:*	none	ECOG Performance Status	
		☐ 0 ☒ 1 ☐ 2 ☐ 3 ☐ 4	

Clinical Data:

BX:
ER: PR: HER2: KI67:



History:
prem left breast cancer ct4 n2 m1 met to liver bone idc2 er5/8 pr8/8
her23+ initial pet ct bilobar hfl largest seg 5 suv 21 m 3.6x3.3 multiple
osseous lesions vertebral few ,rt iliac bone rt adnexal cyst suv 6 but mri
pelvis free..

pt rec 24 weeks taxol and 7 cycles herceptin
cts 29/8/2026 rd of bilobar hfl largest 18x11 comp to 36x33 ,sd of rt iliac
l1 l2 vb tiny lesions rd of left breast mass . for ,mdt to st ht with herceptin

Sonomammography:

Biomarker/Molecular

Metis Workup:

Recommendations:

MDT Outcome & Plan

For Herceptin + Femara + Zoladex
Zometa/3m after dental evaluation

SIGNATURE

MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY
د نظري حسن	د لطيفة عبدوي		

Signature of Dr. Hassan Nazzari (Medical Oncology) and Dr. Laila Abdel-Wahab (Surgery) dated 30/9/2016.